

File Information		Artwork Type [PLEASE TICK ✓]		Sign / Stamp	
Product : T VYTOCOR 10MG-20MG TABLET [FRONT] SAP No. : Version : RASUB/V1 Date : 05 July 2024		☐ Unit Box ☐ Outer Box ☐ Label		☒ Leaflet ☐ Fix-A-Form ☐ Others : _____ ☐ Aluminium Foil ☐ Sachet	
Document Category [PLEASE TICK ✓]		Pharmacode / QR Code / Others			
☒ RA Submission Artwork ☐ Internal Artwork Circulation ☐ Others : _____		☐ NPRA Approval Copy ☐ Previous Approved Artwork		☐ Approval Copy Artwork ☐ Draft Artwork	
Remarks		Colour			
Font : Arial Narrow (Spacing - 6pt / Size - 6pt) Dimension : 210mm(w) x 297mm(h)		Black 100% Colours shown on this proof are to indicate correct colour separations.			
Pages No : 1 of 2					



DESCRIPTION: Vytocor 10mg/20mg Tablet: White to off-white, capsule shaped, with '20' marking on one side and '10' on the other side, tablet

COMPOSITION: Each tablets contains 20mg Simvastatin and 10mg Ezetimibe.

PHARMACODYNAMICS

VYTOCOR (ezetimibe/simvastatin) is a lipid-lowering product that selectively inhibits the intestinal absorption of cholesterol and related plant sterols and inhibits the endogenous synthesis of cholesterol.

Mechanism of Action

VYTOCOR contains ezetimibe and simvastatin, two lipid-lowering compounds with complementary mechanism of action. VYTOCOR reduces elevated total-C, LDL-C, APO B, TG, and non-HDL-C, and increases HDL-C through dual inhibition of cholesterol absorption and synthesis.

Ezetimibe

Ezetimibe inhibits the intestinal absorption of cholesterol. Ezetimibe is orally active and has a mechanism of action that differs from other classes of cholesterol-reducing compounds (e.g., statins, bile acid sequestrants [resins], fibric acid derivatives, and plant stanols). The molecular target of ezetimibe is the sterol transporter, Niemann-Pick C1-Like 1 (NPC1L1), which is responsible for the intestinal uptake of cholesterol and phytosterols. Ezetimibe localizes at the brush border of the small intestine and inhibits the absorption of cholesterol, leading to a decrease in the delivery of intestinal cholesterol to the liver; statins reduce cholesterol synthesis in the liver and together these distinct mechanisms provide complementary cholesterol reduction.

Simvastatin

After oral ingestion, simvastatin, which is an inactive lactone, is hydrolyzed in the liver to the corresponding active β -hydroxyacid form which has a potent activity in inhibiting HMG-CoA reductase (3 hydroxy-3 methylglutaryl CoA reductase). This enzyme catalyses the conversion of HMG- CoA to mevalonate, an early and rate -limiting step in the biosynthesis of cholesterol. Simvastatin has been shown to reduce both normal and elevated LDL-C concentrations. LDL is formed from very-low-density protein (VLDL) and is catabolized predominantly by the high affinity LDL receptor. The mechanism of the LDL-lowering effect of simvastatin may involve both reduction of VLDL-cholesterol (VLDL-C) concentration and induction of the LDL receptor, leading to reduced production and increased catabolism of LDL-C. Apolipoprotein B also falls substantially during the treatment with simvastatin. In addition, simvastatin moderately increases HDL-C and reduces plasma TG. As a result of these changes, the ratios of total - to HDL-C and LDL-to HDL-C are reduced.

PHARMACOKINETICS

Absorption

Ezetimibe

After oral administration, ezetimibe is rapidly absorbed and extensively conjugated to a pharmacologically active phenolic glucuronide (ezetimibe-glucuronide). Mean maximum plasma concentrations (C_{max}) occur within 1 to 2 hours for the ezetimibe-glucuronide and 4 to 12 hours for ezetimibe. The absolute bioavailability of ezetimibe cannot be determined as the compound is virtually insoluble in aqueous media suitable for injection. Concomitant food administration (high fat or non-fat meals) had no effect on the oral bioavailability of ezetimibe when administered as ezetimibe 10 mg tablets.

Simvastatin

The availability of β -hydroxyacid to the systemic circulation following an oral dose of simvastatin was found to be less than 5% of the dose, consistent with extensive hepatic first-pass extraction. The major metabolites of simvastatin present in human plasma are the β -hydroxyacid and four additional active metabolites. Relative to the fasting state, the plasma profiles of both active and total inhibitors were not affected when simvastatin was administered immediately before a test meal.

Distribution

Ezetimibe

Ezetimibe and ezetimibe-glucuronide are bound 99.7% and 88 to 92% to human plasma proteins, respectively.

Simvastatin

Both simvastatin and the β -hydroxyacid are bound to human plasma proteins (95%).

The pharmacokinetics of single and multiple doses of simvastatin showed that no accumulation of drug occurred after multiple dosing. The maximum plasma concentration of inhibitors occurred 1.3 to 2.4 hours post-dose.

Metabolism

Ezetimibe

Ezetimibe is metabolized primarily in the small intestine and liver via glucuronide conjugation (a phase (II) reaction) with subsequent biliary excretion. Minimal oxidative metabolism (a phase (I) reaction) has been observed in all species evaluated. Ezetimibe and ezetimibe-glucuronide are the major drug-derived compounds detected in plasma, constituting approximately 10 to 20% and 80 to 90 % of the total drug in plasma, respectively. Both ezetimibe and ezetimibe-glucuronide are slowly eliminated for plasma with evidence of significant enterohepatic recycling. The half-life for ezetimibe and ezetimibe-glucuronide is approximately 22 hours.

Simvastatin

Simvastatin is an inactive lactone which is readily hydrolysed in vivo to the corresponding β -hydroxyacid, a potent inhibitor of HMG-CoA reductase. Hydrolysis takes place mainly in liver, the rate of hydrolysis in human plasma is very slow.

In man, simvastatin is well absorbed and undergoes extensive hepatic first-pass extraction. The extraction in the liver is dependent on the hepatic blood flow. The liver is its primary site of action, with subsequent excretion of drug equivalents in the bile. Consequently, availability of active drug to the systemic circulation is low.

Following an intravenous injection of the β -hydroxyacid metabolite, its half-life averaged 1.9 hours.

Elimination

Ezetimibe

Following oral administration of 14 C-ezetimibe (20mg) to human subjects, total ezetimibe accounted for approximately 93 % of the total radioactivity in plasma. Approximately 78% and 11% of the administered radioactivity were recovered in the faeces and urine, respectively, over a 10-day collection period. After 48 hours, there were no detectable levels of radioactivity in the plasma.

Simvastatin

Following an oral dose of radioactive simvastatin to man, 13% of the radioactivity was excreted in the urine and 60% in the faeces within 96 hours. The amount recovered in the faeces represents absorbed drug equivalents excreted in bile as well as unabsorbed drug. Following an intravenous injection of the β -hydroxyacid metabolite, an average of only 0.3% of the IV dose was excreted in urine as inhibitors.

INDICATION

Prevention of cardiovascular events

VYTOCOR is indicated to reduce the risk of cardiovascular events (cardiovascular death, nonfatal myocardial infarction, nonfatal stroke, hospitalization for unstable angina, or need for revascularization), in patients with coronary heart disease (CHD) and history of acute coronary syndrome (ACS), either previously treated with a statin or not.

Primary Hypercholesterolemia

VYTOCOR is indicated as adjunctive therapy to diet for the reduction of elevated total cholesterol (total-C), low density lipoprotein cholesterol (LDL-C), apolipoprotein B (Apo B), triglycerides (TG), and non-high density lipoprotein cholesterol (non-HDL-C), and to increase high-density lipoprotein cholesterol (HDL-C) in patients with primary (heterozygous familial and non-familial) hypercholesterolemia or mixed hyperlipidaemia.

Homozygous Familial Hypercholesterolemia (HoFH)

VYTOCOR is indicated for the reduction of elevated total-C and LDL-C levels in patients with HoFH. Patients may also receive adjunctive treatments (e.g., LDL apheresis).

DOSAGE AND ADMINISTRATION

The patient should be placed on a standard cholesterol-lowering diet before receiving VYTOCOR and should continue on this diet during treatment with VYTOCOR. The dosage should be individualized according to the baseline LDL-C level, the recommended goal of therapy, and the patient's response. VYTOCOR should be taken as a single daily dose in the evening, with or without food.

The 80mg dose is only recommended in patients at high risk for cardiovascular complications who have not achieved treatment goals on lower doses and when the benefits are expected to outweigh the potential risks.

Patients with Primary Hypercholesterolemia

In patients with primary hyperlipidemia or mixed hyperlipidemia, the dosage range is 10/10 mg/day through 10/80 mg/day. The recommended usual starting dose is 10/20 mg/day. Initiation of therapy with 10/10 mg/day may be considered for patients requiring less aggressive LDL-C reductions. Patients who require a larger reduction in LDL-C (greater than 55%) may be started at 10/40 mg/day. After initiation or titration of VYTOCOR, lipid levels may be analysed after 2 or more weeks and dosage adjusted, if needed. The 10/80 mg dose of VYTOCOR is only recommended in patients at high risk for cardiovascular complications who have not achieved their treatment goals on lower doses and when the benefits are expected to outweigh the potential risks.

For patients who are currently on both simvastatin and ezetimibe, no dosage adjustment is required when switching to VYTOCOR.

Patients with Coronary Heart Disease

In the cardiovascular events risk reduction study (IMPROVE-IT), the starting dose was 10/40 mg once a day in the evening. The 10/80-mg dose is only recommended when the benefits are expected to outweigh the potential risks.

Patients with Homozygous Familial Hypercholesterolemia

The recommended dosage for patients with homozygous familial hypercholesterolemia is VYTOCOR 10/40mg/day or 10/80mg/day in the evening. The 10/80mg dose is only recommended when the benefits are expected to outweigh the potential risks. VYTOCOR should be used as an adjunct to other lipid-lowering treatments (e.g., LDL apheresis) in these patients or if such treatments are unavailable.

In patients taking lomitapide concomitantly with VYTOCOR, the dose of VYTOCOR should not exceed 10/40mg/day

Use in the Elderly

No dosage adjustment is required for elderly patients.

Use in Paediatric Patients

Treatment with VYTOCOR is **NOT RECOMMENDED**.

Use in Hepatic Impairment

No dosage adjustment is required in patients with mild hepatic insufficiency (Child-Pugh score 5 or 6). Treatment with VYTOCOR is not recommended in patients with moderate (Child-Pugh score 7 to 9) or severe (Child-Pugh score >9) liver dysfunction

Use in Renal Impairment

No dosage adjustment is required for patients with moderate renal insufficiency. If treatment in patients with severe renal insufficiency (creatinine clearance $\leq$ 30mL/min) is deemed necessary, dosages above 10/10mg/day should be implemented cautiously.

Co-administration with other medicines

Co-administration with other medicines/ Concomitant Therapy

Dosing of VYTOCOR should occur either $\geq$ 2 hours before or $\geq$ 4 hours after administration of a bile acid sequestrant.

In patients taking products containing elbasvir or grazoprevir concomitantly with VYTOCOR, the dose of VYTOCOR should not exceed 10/20 mg/day.

In patients taking fibrates (other than gemfibrozil and fenofibrate) concomitantly with VYTOCOR, the dose of VYTOCOR should not exceed 10mg/day of simvastatin.

However, the safety and effectiveness of VYTOCOR administered with fibrates have not been studied. Therefore, the combination of VYTOCOR and fibrates should be avoided.

In patients taking amiodarone, verapamil or diltiazem concomitantly with VYTOCOR, the dose of VYTOCOR should not exceed 20mg/day of simvastatin.

In patients taking amlodipine or lipid-lowering dose of niacin ($\geq$ 1g/day) concomitantly with VYTOCOR, the dose of VYTOCOR should not exceed 40mg/day of simvastatin.

ROUTE OF ADMINISTRATION: Oral

CONTRAINDICATIONS

- Hypersensitivity to the active substances or to any of the excipients.
- Active liver disease or unexplained persistent elevations of serum transaminases.
- Pregnancy and nursing
- Concomitant administration of potent CYP3A4 inhibitors (e.g., itraconazole, ketoconazole, posaconazole, voriconazole, HIV protease inhibitors, boceprevir, telaprevir, erythromycin, clarithromycin, telithromycin, nefazodone)
- Concomitant administration of gemfibrozil, cyclosporine, or danazol.

WARNING(s) AND PRECAUTION(s)

Myopathy/Rhabdomyolysis

Simvastatin, like other inhibitors of HMG-CoA reductase, occasionally causes myopathy manifested as muscle pain, tenderness, or weakness with creatine kinase (CK) above 10X the upper limit of normal (ULN). Myopathy sometimes takes the form of rhabdomyolysis with or without acute renal failure secondary to myoglobinuria, and rare fatalities have occurred. The risk of myopathy is increased by high levels of HMG-CoA reductase inhibitory activity in plasma, which may be due, in part, to interacting drugs that interfere with simvastatin metabolism and/or transporter pathways. Predisposing factors for myopathy include advanced age ($\geq$ 65 years), female gender, uncontrolled hypothyroidism, and renal impairment.

The risk of myopathy is greater in patients on simvastatin 80mg compared with other statin-based therapies with similar LDL-C lowering efficacy. Therefore, 10/80mg dose of VYTOCOR should only be used in patients at high risk for cardiovascular complications who have not achieved their treatment goals on lower doses and when the benefits are expected to outweigh the potential risks. In patients taking VYTOCOR 10/80mg for whom an interacting agent is needed, a lower dose of VYTOCOR or an alternative statin- ezetimibe regimen with less potential for drug-drug interactions should be used.

All patients starting therapy with VYTOCOR, or whose dose of VYTOCOR is being increased, should be advised of the risk of myopathy and told to report promptly and unexplained muscle pain, tenderness or weakness. VYTOCOR therapy should be discontinued immediately if myopathy is diagnosed or suspected.

The presence of these symptoms, and a CK level >10 times the upper limit of normal indicates myopathy. In most cases, when patients were promptly discontinued from simvastatin treatment, muscle symptoms and CK increases resolved. Periodic CK determinations may be considered in patients starting therapy with VYTOCOR or whose dose is being increased. Periodic CK determinations are recommended for patients titrating to the 10/80mg dose. There is no assurance that such monitoring will prevent myopathy.

Many of the patients who have developed rhabdomyolysis on therapy with simvastatin have had complicated medical histories, including renal insufficiency usually as a consequence of long-standing diabetes mellitus. Such patients taking VYTOCOR merit closer monitoring. Therapy with VYTOCOR should be temporarily stopped a few days prior to elective major surgery and when any major medical or surgical condition supervenes.

There have been very rare reports of an immune-mediated necrotizing myopathy (IMNM) during or after treatment with some statins. IMNM is clinically characterized by:

- Persistent proximal muscle weakness and elevated serum creatine kinase, which persist despite discontinuation of statin treatment
- Muscle biopsy showing necrotizing myopathy without significant inflammation
- Improvement with immunosuppressive agents

Because VYTOCOR contains simvastatin, the risk of myopathy/rhabdomyolysis is increase by concomitant use of VYTOCOR with following drugs:

Contraindicated Drugs

- Potent inhibitors of CYP3A4
- Gemfibrozil, cyclosporine, or danazol

Other Drugs

- Fusidic acid
- Amiodarone
- Calcium Channel Blockers (verapamil/diltiazem, Amlodipine)
- Lomitapide
- Moderate inhibitors of CYP3A4
- Inhibitors of Breast Cancer Resitant Protein (BCRP)
- Other Fibrates
- Niacin ($\geq$ 1g/day)
- Daptomycin
- Anticoagulants

File Information		Artwork Type [PLEASE TICK ✓]		Sign / Stamp	
Product : T VYTOCOR 10MG-20MG TABLET [BACK] SAP No. : Version : RASUB/V1 Date : 05 July 2024		☐ Unit Box ☐ Outer Box ☐ Label		☒ Leaflet ☐ Fix-A-Form ☐ Others : _____ ☐ Aluminium Foil ☐ Sachet	
Document Category [PLEASE TICK ✓]		Pharmacode / QR Code / Others			
☒ RA Submission Artwork ☐ Internal Artwork Circulation ☐ Others : _____		☐ NPRA Approval Copy ☐ Previous Approved Artwork		☐ Approval Copy Artwork ☐ Draft Artwork	
Remarks		Colour			
Font : Arial Narrow (Spacing - 6pt / Size - 6pt) Dimension : 210mm(w) x 297mm(h)		Black 100% Colours shown on this proof are to indicate correct colour separations.			
Pages No : 2 of 2					

Myasthenia Gravis/Ocular Myasthenia

In few cases, statins have been reported to induce new onset or aggravate pre-existing myasthenia gravis or ocular myasthenia. VYTOCOR should be discontinued in case these conditions occur. There have been reports of recurrences of these conditions when the same or a different statin was (re-) administered.

Liver Enzymes

It is recommended that LFTs be performed before treatment with VYTOCOR begins and thereafter when clinically indicated. Patients titrated to 10/80mg dose should receive an additional test prior to titration, 3 months after titration to the 10/80mg dose, and periodically thereafter (e.g., semi-annually) for the first year of treatment. Special attention should be paid to patients who develop elevated serum transaminase levels, and in these patients, measurements should be repeated promptly and then performed more frequently. If the transaminase levels show evidence of progression, particularly if they rise 3X ULN and are persistent, the drug should be discontinued. Note that ALT may emanate from muscle, therefore ALT rising with CK may indicate myopathy

There have been rare reports of fatal and non-fatal hepatic failure in patients taking statins, including simvastatin. If serious liver injury with clinical symptoms and/or hyperbilirubinemia or jaundice occurs during treatment with VYTOCOR, promptly interrupt therapy. If an alternate aetiology is not found, do not restart VYTOCOR.

VYTOCOR should be used with caution in patients who consume substantial quantities of alcohol and/or have a past history of liver disease. Active liver disease or unexplained persistent transaminase elevations are contraindications to the use of VYTOCOR.

Hepatic Insufficiency

Due to the unknown effects of the increased exposure to ezetimibe in patients with moderate or severe hepatic insufficiency, VYTOCOR is not recommended in these patients

DRUG INTERACTIONS

No clinically significant pharmacokinetics interaction was seen when ezetimibe was co-administered with simvastatin. VYTOCOR is bioequivalent to co-administered ezetimibe and simvastatin.

Contraindicated Drugs

Concomitant use of the following drugs is **contraindicated**:-

• **Potent inhibitors of CYP3A4**

No clinically significant pharmacokinetic interactions have been observed between ezetimibe and drugs known to be metabolised by cytochromes P450 1A2, 2D6, 2C8, 2C9, and 3A4, or N-acetyltransferase. Simvastatin is metabolised by CYP3A4 but has no CYP3A4 inhibitory activity. Therefore, simvastatin is not expected to affect plasma concentrations of other drugs metabolised by CYP3A4.

Potent inhibitors of CYP3A4: Concomitant use with medicines labelled as having potent inhibitory effect on CYP3A4 at therapeutic doses (e.g.: itraconazole, ketoconazole, posaconazole, voriconazole, erythromycin, clarithromycin, telithromycin, HIV protease inhibitors, boceprevir, telaprevir or nefazodone) is contraindicated. If treatment with potent CYP3A4 inhibitors is unavoidable, therapy with simvastatin should be suspended during the course of treatment.

• **Gemfibrozil, Ciclosporin, or Danazol**

Concomitant use of these drugs with simvastatin is contraindicated

Other drug interactions

• **Other fibrates**

The dose of VYTOCOR should not exceed 10/10mg daily in patients receiving concomitant medication with fibrates other than gemfibrozil or fenofibrate. When simvastatin and fenofibrate are given concomitantly, there is no evidence that the risk of myopathy exceeds the sum of the individual risks of each agent. Caution should be used when prescribing fenofibrate with simvastatin, as either agent can cause myopathy when given alone. Addition of fibrates to simvastatin typically provides little additional reduction in LDL-C, but further reductions of TG and further increases in HDL-C may be obtained. Combinations of fibrates with simvastatin have been used without myopathy in small short-term clinical studies with careful monitoring

Concurrent use of fibrates may cause severe myositis and myoglobinuria.

• **Fusidic Acid**

The risk of myopathy including rhabdomyolysis may be increased by concomitant administration of fusidic acid

• **Amiodarone**

In a clinical trial, myopathy was reported in 6% of patients receiving simvastatin 80mg and amiodarone. The dose of VYTOCOR should not exceed 10/20mg daily in patients receiving concomitant medication with amiodarone

• **Cholestyramine**

Concomitant cholestyramine administration decreased the mean area under the curve (AUC) of total ezetimibe (ezetimibe + ezetimibe glucuronide) approximately 55%. The incremental LDL-C reduction due to adding VYTOCOR to cholestyramine may be lessened by this interaction.

• **Calcium Channel Blockers**

Verapamil or diltiazem: In a clinical trial, patients on diltiazem treated concomitantly with simvastatin 80mg had an increased risk of myopathy. The dose of VYTOCOR should not exceed 10/20mg daily in patients receiving concomitant medication with verapamil or diltiazem.

Amlodipine: In a clinical trial, patients on amlodipine treated concomitantly with simvastatin 80mg had a slightly increased risk of myopathy. The dose of VYTOCOR should not exceed 10/40mg daily in patients receiving concomitant medication with amlodipine.

Niacin (≥1g/day): The dose of VYTOCOR should not exceed 10/40mg daily in patients receiving concomitant medication with niacin (nicotinic acid) ≥1g/day. Cases of myopathy/rhabdomyolysis have been observed with simvastatin co-administered with lipid-modifying doses (≥ 1 g/day) of niacin.

The risk of myopathy/rhabdomyolysis is increased by concomitant administration of verapamil, diltiazem, or amlodipine

• **Lomitapide**

The risk of myopathy and rhabdomyolysis may be increased by concomitant administration of lomitapide with simvastatin.

• **Moderate Inhibitors of CYP3A4**

Patients taking other medicines labelled as having a moderate inhibitory effect on CYP3A4 concomitantly with VYTOCOR, particularly higher VYTOCOR doses, may have an increased risk of myopathy.

• **Inhibitors of the Transport Protein OATP1B1**

Simvastatin acid is a substrate of the transport protein OATP1B1. Concomitant administration of medicinal products that are inhibitors of the transport protein OATP1B1 may lead to increased plasma concentrations of simvastatin acid and an increased risk of myopathy

• **Inhibitors of Breast Cancer Resistant Protein (BCRP)**

Simvastatin is a substrate of the Breast Cancer Resistant Protein (BCRP) efflux transporter. Concomitant administration of products that are inhibitors of BCRP (e.g., elbasvir and grazoprevir) may lead to increased plasma concentrations of simvastatin and an increased risk of myopathy. When co-administering simvastatin with an inhibitor of BCRP, a dose adjustment of VYTOCOR may be necessary.

• **Colchicine**

There have been reports of myopathy and rhabdomyolysis with the concomitant administration of colchicine and VYTOCOR, in patients with renal impairment. Close clinical monitoring of such patients taking this combination is advised.

• **Daptomycin**

The risk of myopathy and/or rhabdomyolysis may be increased by concomitant administration of HMG-CoA reductase inhibitors and daptomycin

Other interactions

• **Grapefruit Juice**

Grapefruit Juice contains one or more components that inhibit CYP3A4 and can increase the plasma concentrations of drugs metabolized by CYP3A4. Intake of grapefruit juice during treatment with VYTOCOR should therefore be avoided.

• **Anticoagulants**

In patients taking coumarin anticoagulants, prothrombin time should be determined before starting VYTOCOR and frequently enough during early therapy to ensure that no significant alteration of prothrombin time occurs. Once a stable prothrombin time has been documented, prothrombin times can be monitored at the intervals usually recommended for patients on coumarin anticoagulants. If the dose of VYTOCOR is changed or discontinued, the same procedure should be repeated. Simvastatin therapy has not been associated with bleeding or with changes in prothrombin time in patients not taking anticoagulants. The effect of VYTOCOR on the prothrombin time has not been studied.

• **Antacids**

Concomitant antacid administration decreased the rate of absorption of ezetimibe but had no effect on the bioavailability of ezetimibe. This decreased rate of absorption is not considered clinically significant.

PREGNANCY AND LACTATION

Atherosclerosis is a chronic process, and ordinarily discontinuation of lipid-lowering drugs during pregnancy should have little impact on the long-term risk associated with primary hypercholesterolemia.

Use in Pregnancy: VYTOCOR is contraindicated during pregnancy.

Use in Lactation: It is not known whether the active components of VYTOCOR are excreted into human breast milk; therefore, women who are nursing should not take VYTOCOR.

SIDE EFFECTS

VYTOCOR is generally well tolerated.

The following common (≥1/100, <1/10) or uncommon (≤1/1000, <1/100); drug-related adverse experiences were reported in patients taking ezetimibe/simvastatin and at a greater incidence than placebo.

Investigations:

Common: ALT and/or AST increased; blood CK increased
Uncommon: blood bilirubin increased; blood uric acid increased; gamma-glutamyl transferase increased; international normalized ratio increased; protein urine present; weight decreased

Nervous system disorders:

Uncommon: dizziness; headache

Gastrointestinal disorders

Uncommon: abdominal pain; abdominal discomfort; abdominal pain upper; dyspepsia; flatulence; nausea; vomiting

Skin and subcutaneous tissue disorders

Uncommon: pruritus; rash

Musculoskeletal and connective tissue disorders

Uncommon: arthralgia; muscle spasms; muscular weakness; musculoskeletal discomfort; neck pain; pain in extremity

General disorders and administration site conditions

Uncommon: asthenia; fatigue; malaise; oedema peripheral

Psychiatric disorders

Uncommon: sleep disorders

The following common (≥1/100, <1/10) or uncommon (≤1/1000, <1/100); drug-related adverse experiences were reported in patients taking ezetimibe/simvastatin and at a greater incidence than statins administered alone.

Investigations:

Common: ALT and/or AST increased
Uncommon: blood bilirubin increased; blood CK increased; gamma-glutamyl transferase increased

Nervous system disorders:

Uncommon: headache; paraesthesia

Gastrointestinal disorders

Uncommon: abdominal distension; diarrhoea; dry mouth; dyspepsia; flatulence; gastroesophageal reflux disease; vomiting

Skin and subcutaneous tissue disorders

Uncommon: pruritus; rash; urticaria

Musculoskeletal and connective tissue disorders

Common: myalgia
Uncommon: arthralgia; back pain; muscle spasms; muscular weakness; musculoskeletal pain; pain in extremity
Frequency not known: Immune-mediated necrotizing myopathy

General disorders and administration site conditions

Uncommon: asthenia; chest pain; fatigue; oedema peripheral

Psychiatric disorders

Uncommon: insomnia

There have been rare post-marketing reports of cognitive impairment (e.g. memory loss, forgetfulness, amnesia, memory impairment, confusion) associated with statin use. These cognitive issues have been reported for all statins. The reports are generally non-serious and reversible upon statin discontinuation, with variable times to symptom onset (1 day to years) and symptom resolution (median 3 weeks). Increases in HbA1c and fasting blood glucose have been reported with statins. The risk of hyperglycemia, however, is outweighed by the reduction in vascular risk with statins.

SYMPTOMS AND TREATMENT OF OVERDOSAGE

No specific treatment of overdosage with VYTOCOR can be recommended. In the event of overdose, symptomatic and supportive measures should be employed.

Ezetimibe

A few cases of overdosage have been reported; most have not been associated with adverse experiences. Reported adverse experiences have not been serious.

Simvastatin

A few cases of overdosage have been reported; the maximum dose taken was 3.6 g. All patients recovered without sequelae.

EFFECTS ON ABILITY TO DRIVE & USE MACHINE

No studies on the effects on the ability to drive and use machines have been performed. However, when driving vehicles or operating machines, it should be taken into account that dizziness has been reported.

STORAGE CONDITION

Store below 30°C.

Keep out of reach of children. Jauhi daripada kanak-kanak.

SHELF LIFE

Please refer to the expiry date on the outer carton.

PACKING / PACK SIZE (S)

Blister packs of 10's per strip into 3 strips per box with packaging insert.

PRODUCT REGISTRATION HOLDER

DUOPHARMA MANUFACTURING (BANGI) SDN. BHD.

Lot No. 2, 4, 6, 8 & 10, Jalan P/7, Section 13, Bangi Industrial Estate, 43650, Bandar Baru Bangi, Selangor Darul Ehsan, Malaysia

MANUFACTURER

DUOPHARMA MANUFACTURING (BANGI) SDN BHD

Lot No. 2 & 4 Jalan P/7, Section 13, Bangi Industrial Estate, 43650, Bandar Baru Bangi, Selangor Darul Ehsan, Malaysia

Revision Date: July 2024

15000XXXX XX

